

Clinical Governance and Authorisation Policy

Responsibility, approval gates, documentation, and human oversight

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NB-GOV-101	1.0	2026-08-01	2027-08-01

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Owner: Medical Director
Approved by: Fictional Clinical Governance Committee
Classification: Training / public synthetic corpus

Document control and RAG metadata

Field	Value
Canonical title	Clinical Governance and Authorisation Policy
Document ID	NB-GOV-101
Keywords	governance, authorization, prescribing, clinician approval, audit, responsibility
Authority	Medical Director
Supersedes	None - initial synthetic edition
Applicability	Training simulations at Northbridge Cardiology Practice only

Retrieval rule

Retrieve for any request to recommend, start, stop, change, renew, dispense, or administer medication, or to produce a final care plan.

1. Roles and accountability

Role	May do	May not do
Cardiologist	Diagnose; authorize medication plan; approve monitoring and follow-up	Delegate final clinical accountability to the agent
Prescribing clinician	Sign a complete order within competence	Sign an incomplete or ambiguous order
Clinical pharmacist	Reconcile medicines; check dose, interactions, allergies, duplication, monitoring	Change therapy without an authorized prescribing pathway
Cardiac nurse	Collect observations; educate from signed plan; escalate	Independently select or alter medicine
Booking coordinator	Book approved appointments and tests	Interpret results or decide clinical urgency
Decision-support agent	Retrieve, compare, flag, draft, cite, and format	Prescribe, diagnose, sign, contact emergency services, or silently resolve conflict

2. Three-gate medication workflow

Gate A - identity and reconciliation

- Match full name, date of birth, patient ID, and at least one additional identifier.
- Confirm active, stopped, held, and patient-reported medicines.
- Record allergies, reactions, severity, renal/hepatic status, pregnancy relevance where applicable, and adherence uncertainties.

Gate B - clinical and pharmaceutical review

- Confirm indication, goals, current observations, recent tests, interactions, duplication, and monitoring due dates.
- A pharmacist or authorized clinician clears unresolved medicine-name, dose, or interaction discrepancies.

Gate C - signed authorization

- The order states generic/brand name where needed, strength, dose, unit, route, frequency, start date, duration or review date, indication, special instructions, monitoring, hold/escalation criteria, and author identity.
- Only after Gate C may the system present the plan as 'authorized'. Before then it must say 'draft for clinician approval'.

3. Required final review format

Section	Required content
Medication recommendation	Continue/start/adjust/hold/stop only as explicitly authorized; include rationale and authorization status
Exact directions	Medicine, formulation, dose, route, frequency, timing, duration, missed-dose source wording, and practical instructions
Warnings	Allergy, interaction, adverse-effect, monitoring, driving, pregnancy, procedure, and sick-day issues where documented
Further tests	Test, due window, booking owner, results owner, and action if not completed
Appointments	Type, urgency, target date/window, prerequisites, and responsible service
Safety net	Symptoms and exact contact route; emergency wording must be explicit
Evidence	Patient record timestamp plus document-level citations

4. Documentation and audit

- Record request ID, user role, patient ID token, source versions, retrieved sections, model version, tool calls, authorization status, and final reviewer.
- Do not copy full names, dates of birth, free-text notes, or complete prompts into external observability platforms.
- Material changes require a new signed version; do not overwrite the audit trail.
- Verbal instructions must be read back and documented by the receiving clinician according to the fictional downtime process.

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